



PAGE 1 – COVER SHEET

Patient: Mohammad Ben

Age: 88

Diagnosis: Dementia, Hip Fracture, COVID-19

Admission Date to Nursing Facility: 12 January 2026

Facility: Lakeview Senior Care Center, Chicago, IL

Floor: 3rd Floor, Geriatric Wing

Primary Care Physician: Dr. Susan Feldman, MD

Attending Surgeon: Dr. Richard Hall, Orthopedic Surgeon

Insurance: Medicaid – \$12,000/month coverage



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PAGE 3 – PATIENT SUMMARY

Mohammad Ben is an 88-year-old male with a history of moderate-to-severe dementia. He is widowed, lives alone, and was admitted to Lakeview Senior Care Center due to progressive cognitive decline and increasing need for 24-hour supervision. Shortly after admission, he experienced a fall resulting in a left hip fracture. His hospitalization and rehabilitation were complicated by COVID-19 infection approximately two months later.



PAGE 4 – ADMISSION INFORMATION

- **Admission Date:** 12 January 2026
 - **Reason:** Advanced dementia, supervision needs
 - **Floor:** 3rd floor, Geriatric Wing
 - **Room:** 312B
 - **Primary Nurse:** Nurse Emily Roberts
 - **Secondary Nurse:** Nurse Michael Tran
 - **Insurance:** Medicaid – coverage \$12,000/month
 - **Family Contact:** None; patient is widowed
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PAGES 5–6 – MEDICAL HISTORY

- **Dementia:** Moderate to severe, diagnosed 2019
 - **Hypertension:** Diagnosed 2015, controlled
 - **Type 2 Diabetes:** Diagnosed 2012
 - **Cardiac History:** Mild LVH, prior ECG 2023 within normal limits
 - **Fall History:** Multiple minor falls prior to admission
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PAGE 7 – MEDICATION LIST (As of Jan 2026)

- Donepezil 10 mg daily (dementia)
 - Metformin 500 mg twice daily (diabetes)
 - Lisinopril 20 mg daily (hypertension)
 - Acetaminophen 500 mg PRN (pain)
 - Vitamin D 1000 IU daily
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PAGE 8 – INCIDENT REPORTS

- **Date:** 15 February 2026
 - **Event:** Patient fell in room while attempting to walk without assistance
 - **Injury:** Left hip fracture (intertrochanteric)
 - **Response:** Transferred to Northwestern Memorial Hospital
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PAGE 9 – HOSPITAL TRANSFERS & SURGICAL INTERVENTION

Hospital: Northwestern Memorial Hospital, Chicago, IL

- **Admission:** 15 February 2026
 - **Surgery Date:** 16 February 2026
 - **Procedure:** Open Reduction and Internal Fixation (ORIF) of left hip
 - **Surgeon:** Dr. Richard Hall
 - **Surgical Time:** 1 hour 45 minutes
 - **Outcome:** Initial fixation successful but post-op complications included delirium and poor mobilization
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PAGE 10 – ECG AND VITAL SIGNS

Admission ECG (15 Feb 2026):

- Rate: 88 bpm
- Rhythm: Sinus rhythm
- QRS: 0.09 sec
- QTc: 440 ms
- Notable: Left ventricular hypertrophy

Vital Signs (daily, first week post-surgery):

- Temp: 98.6–100.2 °F
 - BP: 130/70 mmHg
 - HR: 88–95 bpm
 - O2 Sat: 94–96%
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PAGE 11 – REHABILITATION PLAN (Hip)

- **Start Date:** 18 February 2026
 - **Plan:**
 - Bedside mobilization day 1–3 post-surgery
 - Physical therapy 30 min daily
 - Gradual transfer to walker-assisted standing
 - Goal: Ambulate 50 feet with walker within 2 weeks
 - **Outcome:** Patient tolerated initial therapy but showed progressive fatigue and delirium, limiting rehab success
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PAGE 12 – NURSING NOTES (Selected)

- **Emily Roberts (Nurse, 3rd floor)** – noted confusion, agitation, incontinence
 - **Michael Tran (Nurse, 3rd floor)** – assisted with daily transfers, reported mild wound drainage, monitored vitals
 - **Observation:** Patient required full assistance for all ADLs (activities of daily living) by 1 March 2026
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PAGE 13 – THERAPY LOGS

- PT sessions 3x/week: limited progress
 - OT sessions: focus on ADL retraining
 - Notes: Patient fatigues quickly, limited hip flexion at 30° by week 3 post-op
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PAGE 14 – INFECTION HISTORY

- **COVID-19 Positive:** 10 March 2026
 - **Symptoms:** Fever 101 °F, mild cough, fatigue
 - **Isolation:** 2 weeks, supportive care
 - **Impact:** Rehab suspended for 2 weeks
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PAGE 15 – NUTRITION AND DAILY ROUTINE

- **Diet:** Soft diet, high protein, diabetic restrictions
- **Feeding Assistance:** Required 100%

- **Daily Routine:** Morning vitals, breakfast, PT, lunch, nap, evening activities, bedtime
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PAGE 16 – INSURANCE BILLING SUMMARY

- Medicaid pays \$12,000/month for nursing home stay
 - Additional charges during hospitalization and rehab:
 - PT/OT sessions: \$3,200
 - Blood tests: \$450
 - Imaging (X-ray, CT): \$1,100
 - Medications: \$300
 - Bed rental during hospital stay: \$500/day for 5 days → \$2,500
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PAGE 17 – LABORATORY RESULTS

- **CBC (17 Feb 2026):** Mild anemia, Hgb 11.2 g/dL
 - **BMP (17 Feb 2026):** Na 138, K 4.2, Cr 1.1
 - **COVID PCR (10 Mar 2026):** Positive
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PAGE 18 – IMAGING REPORTS

- **X-ray (15 Feb 2026):** Left intertrochanteric hip fracture
 - **CT scan (15 Feb 2026):** Confirmed fracture, no head trauma
 - **Post-op X-ray (17 Feb 2026):** Hardware in place, minimal displacement
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PAGE 19 – OUTCOMES AND PROGNOSIS

- Hip rehab partially unsuccessful; patient remains wheelchair-bound
 - Dementia progression continues
 - COVID infection resolved without major complications
 - Prognosis: Palliative-focused care with ongoing rehab support and nursing assistance
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PAGE 20 – STAFF INFORMATION AND APPENDICES

- **Primary Nurse:** Emily Roberts
- **Secondary Nurse:** Michael Tran
- **Attending Surgeon:** Dr. Richard Hall, ORIF procedure
- **Primary Physician:** Dr. Susan Feldman, MD
- **Appendices:**
 - A: Hospital Discharge Summary
 - B: Surgery Report
 - C: Therapy Logs
 - D: Insurance Claims Statements
 - E: Lab and Imaging Reports
 - F: COVID-19 Isolation Notes